

Clinical Report - Consultation #4101

Patient: Ava Miller	Date: 2026-05-18 14:58
Clinician: Ada Demo	Visit Type: General

MEDICAL OUTPATIENT CLINICAL REPORT

Facility Name: OPD-Vertex
Department: Outpatient Department (OPD)
Encounter ID: 4101
Date of Visit: 18-May-2026
Time of Visit: 14:58
Clinician: Ada Demo
Report Status: Draft
Source: AI-assisted documentation generated from local transcription and clinician review

1. PATIENT INFORMATION

Full Name: Ava Miller
Age: 38
Gender: F
Date of Birth: 1988-05-11
Patient ID / MRN: 5101
Phone / Contact: +1 555 0101
Address: Not mentioned

2. ENCOUNTER DETAILS

Visit Type: Outpatient follow-up
Mode of Consultation: In person
Accompanied By: Not mentioned
Primary Language: Not mentioned
Information Reliability: Patient history considered reliable

3. CHIEF COMPLAINT

Sore throat, mild fever, and dry cough for two days.

4. HISTORY OF PRESENT ILLNESS

The patient reports sore throat for two days with temperature up to 37.8 C, mild dry cough, painful swallowing, and no shortness of breath. Symptoms started gradually and are worse in the morning. She is able to drink fluids.

5. RELEVANT REVIEW OF SYSTEMS

General: Mild fever, reduced appetite, no chills.
Respiratory: Mild dry cough, no dyspnea, no wheezing.
Cardiovascular: No chest pain, no palpitations.
Gastrointestinal: No nausea, no vomiting, no diarrhea.
Neurological: No headache, no focal weakness.
Genitourinary: No urinary symptoms.
Musculoskeletal: Mild body aches.

Other Systems: No sick contacts confirmed.

6. PAST MEDICAL HISTORY

Mild seasonal allergies. No chronic cardiopulmonary disease.

7. MEDICATION HISTORY

Current Medications Mentioned:

- Cetirizine as needed for allergies.

Recent Over-the-Counter / Home Remedies:

- Not mentioned

Medication Adherence Notes:

Not mentioned

8. ALLERGIES

Drug Allergies: No known drug allergies.

Food Allergies: Not mentioned

Other Allergies: Not mentioned

Reaction Details: Not mentioned

9. FAMILY AND SOCIAL HISTORY

Family History: Mother with allergic rhinitis.

Smoking Status: Never smoker.

Alcohol Use: Occasional social alcohol use.

Substance Use: Denies recreational drug use.

Occupation / Exposure Risks: Primary school teacher.

Pregnancy / Lactation Status: Not mentioned

Other Social Factors: Not mentioned

10. VITAL SIGNS

Blood Pressure: 112/70 mmHg

Heart Rate: 84 bpm

Temperature: 37.8 C

Respiratory Rate: 16/min

SpO2: 99% on room air

Weight: 64 kg

Height: 167 cm

BMI: 22.9

11. PHYSICAL EXAMINATION / EXAMINATION FINDINGS

General Appearance:

Not mentioned

Systemic Examination:

Oropharynx mildly erythematous without exudate, cervical lymph nodes mildly tender, lungs clear to auscultation, patient well hydrated and in no acute distress.

12. CLINICAL ASSESSMENT

Primary Diagnosis / Working Diagnosis:

Acute viral pharyngitis.

Differential Diagnoses:

- Streptococcal pharyngitis
- Early upper respiratory tract infection.

Clinical Impression:

Stable outpatient presentation without red flags. Conservative treatment is appropriate.

13. MANAGEMENT PLAN

A. Medications Prescribed

- Medication Name: Paracetamol
- Dose: 500 mg
Route: oral
Frequency: every 6 to 8 hours as needed
Duration: 3 days
Special Instructions: take after food if stomach upset

B. Investigations Ordered

Laboratory Tests:

- Rapid strep test if fever worsens or throat pain persists beyond 72 hours.

Imaging:

- None.

Procedures:

- Not mentioned

C. Referrals

- None.

D. Follow-up Plan

Re-evaluate in 3 to 5 days if symptoms do not improve.

14. PATIENT INSTRUCTIONS

Rest, hydrate well, use warm saline gargles, and avoid sharing utensils until symptoms improve.

15. SAFETY-NET / RETURN PRECAUTIONS

The patient was advised to seek urgent medical attention if any of the following occur:

- Seek urgent care for difficulty breathing
- inability to swallow fluids
- fever above 39 C
- worsening throat swelling.

16. CLINICAL NOTES SUMMARY

Two-day uncomplicated sore throat syndrome most consistent with viral pharyngitis. Supportive outpatient management discussed.

17. DOCUMENTATION GAPS / MISSING BUT RELEVANT INFORMATION

- Exact exposure to known streptococcal infection not confirmed.

18. INTERNAL CLINICIAN REVIEW FLAGS

(Internal use only - do not include in patient-facing printout unless approved)

Risk Level: Not run

Review Suggestions:

- Not run

19. CLINICIAN APPROVAL

Reviewed By: Not mentioned

Reviewed On: Not mentioned

Final Status: Draft pending clinician approval

Digital Signature / Initials: Not mentioned

20. AUDIT / SYSTEM METADATA

Transcript Source: Faster-Whisper local transcription

Normalization Status: Completed

Report Generation Model: qwen3:8b

Safety Review Status: Not run

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